



Clozapine Prescribing, Administering and Monitoring Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Kalamunda Group Armadale Health Service Armadale Mental Health Service	Mental Health	Medical Nursing Allied Health Administration

2. Purpose

This operational procedure aims to provide clinicians with guidelines to ensure the safe prescription, monitoring and administration of Clozapine to consumers within the in-patient and Community Mental Health Settings

Due to the risk of neutropenia and agranulocytosis, all consumers taking Clozapine are enrolled in a registry and monitored regularly. Protocols for prescribing and monitoring the haematological and physical health of consumers are to be followed. Attention must be paid to any interruption of Clozapine therapy.

3. Background

Clozapine is an effective antipsychotic medication and considered 'gold standard' internationally for the treatment of treatment resistant schizophrenia. However, Clozapine has the potential to cause life-threatening side effects and should only be considered for patients who have not responded to or experienced severe side effects from at least two other antipsychotic medications.

There are 3 brands of Clozapine available in Australia: Clopine® (Pfizer) and Clozaril® (Novartis) and Clozitor® (Pharmacor). Each brand has an associated patient monitoring system:

- ClopineCENTRAL™ for Clopine® available at <https://www.clopine.com.au>
- Clozaril Patient Monitoring System (CPMS) for Clozaril® available at <http://www.ecpms.com.au>
- Juno Connected™ Clozitor® available at <https://www.junoconnected.com.au>.

NB These brands are not interchangeable due to lack of connectivity between the monitoring systems

Clopine® is the current WA Department of Health tendered brand and will be referred to as the default clozapine patient monitoring system."

The medication tender for the state is reviewed periodically, which may affect future prescribing requirements.

Vigilant haematological and physical health monitoring is an ongoing requirement for patients taking Clozapine. This includes regular monitoring of a patient full blood count, monitoring for constipation and gastrointestinal hypomotility and for metabolic and cardiac side effects.

The Australian Therapeutic Goods Administration (TGA) with many other countries around the world mandate regular haematological monitoring and patient tracking to minimise the risk of Clozapine side effects. This is achieved by enrolling every patient taking Clozapine onto a mandatory patient monitoring system (Clopine Central™ for patients prescribed Clopine® and eCPMS™ for patients prescribed Clozaril®).

Both centres offer protocols and product information.

The risk of developing agranulocytosis and neutropenia is closely monitored by mandatory testing and entry of White Cell Count (WCC) and Absolute Neutrophil Count (ANC) onto the patient monitoring system. This is performed:

- Weekly for patients who are initiating Clozapine and within their first 18 weeks of treatment or has had a therapy interruption of more than 4 weeks. (Note the development of agranulocytosis is greatest in this period).
- Every 28 days for patients who have completed their first 18 weeks of therapy.
- Increased monitoring is required if a patients WCC and/or ANC fall into the amber range. (Discussed further under blood tests section).
- Clozapine is initiated while the consumer is an inpatient. They should be transitioned to the community mental health teams for continued monitoring

4. Roles and Responsibilities

Role	Responsibilities
Administration/Clerical	Responsible for booking of medical appointments, faxing/emailing/posting of prescriptions and verified blood results to registered pharmacy. Confirm consumer demographics at each appointment Ensures COVID screening prior to clinic entry
Care Coordinator	Community based clinician who is allocated to the consumer to provide a point of contact with the service and to assist in coordinating aspect of their care To monitor mental health between scheduled medical reviews
Consultant Psychiatrist	Medical lead who provides oversight of the consumers care and treatment with a face-to-face review of the consumer every three - six months (in community) depending upon the complexity of the consumers illness/issues. Register with Clozapine monitoring services
Clozapine clinic	Specialises in the monitoring of consumers prescribed Clozapine in the community and provision of comprehensive mental health and physical health checks.

Role	Responsibilities
Clozapine Nurse	An identified registered nurse is responsible for undertaking physical observations of those attending Assists in obtaining blood results if not available by contacting relevant pathology clinics Monitoring Clopine Central™ and eCPMS™ alerts and communicating to relevant clinicians
Clozapine Coordinator	An identified clinician with registered nurse qualification who coordinates the community clinic and may undertake the role of the Clozapine nurse.
Pharmacy	The consumer's nominated pharmacy who enters the blood results into the Clozapine monitoring system and dispenses the medication against in date blood results
Prescribing Doctor	Allocated medical staff who provides mental state and physical review of the consumer at intervals identified by the consumers current level of monitoring. Responsible for verifying the blood results and ensuring within the range for which a prescription can be issued Register with Clozapine monitoring services

5. Contraindications and Precautions

5.1 Contraindications to commencing Clozapine Therapy

- A history of drug-induced agranulocytosis
- Bone marrow disorders
- Circulatory collapse and/or CNS depression due to any cause
- Alcoholic and other toxic psychoses, drug intoxication, comatose conditions
- Severe renal or cardiac disorders (e.g. myocarditis*, cardiomyopathy)
- Severe hepatic disease including active hepatic disease associated with nausea, anorexia or jaundice, progressive hepatic disease, hepatic failure
- Uncontrolled epilepsy
- Paralytic ileus
- Rechallenge has been successfully completed, however recurrence is possible. Cardiologist advice must be sought on rechallenged with clozapine in patients with previous history of myocarditis.
- Severe hepatic, renal or cardiovascular disorders including active hepatic disease associated with nausea, anorexia or jaundice, progressive hepatic disease and hepatic failure.

5.2 Special Precautions for the use of Clozapine include:

- **Pregnancy:** The decision to prescribe clozapine during pregnancy must be based on thorough evidence based individualised risk benefit analysis by the treating doctor. Liaison with specialist perinatal psychiatric services is recommended and additional monitoring (including therapeutic drug monitoring) during pregnancy is prudent.

- **Breastfeeding:** Since Clozapine is excreted in breast milk, mothers receiving Clozapine should be informed of the possible risks whilst considering the benefits of breastfeeding
- **Older People:** Use in older consumers requires a lower dose at initiation of treatment, and the dose titrated up more slowly as older people are more susceptible to side effects
- **Seizures / Renal / Liver Impairment:** In consumers with a history of seizures, or suffering from cardiovascular, renal or hepatic disorders the initial dose should be 12.5mg given once on the first day, and any dose increase should be slow and in small increments (contact the monitoring centre for dosing advice if required).
- **Caffeine and Smoking:** A change in caffeine and smoking intake can affect Clozapine metabolism and may lead to a rise in Clozapine levels and cause toxicity.
- **Constipation:** Clozapine has potent anticholinergic effects and can result in significant gastrointestinal hypomotility and associated complications if unmanaged. Hospitalisation and deaths have occurred. Patients who have a baseline reduced bowel function have an increased risk of gastrointestinal adverse events when commencing Clozapine. Screening and correcting for constipation prior to/during treatment is essential.
- **Benign Ethnic Neutropenia (BEN):** Ben is an hereditary condition characterised by a low baseline neutrophil count but with no reduction in ability to mount an immune response. There is a higher prevalence of BEN in individuals of African, Jewish, Middle Eastern and Afro-Caribbean descent. Patients with BEN who develop a clozapine induced decrease in the neutrophil count but have no evidence of infection/impaired phagocytosis may usually resume Clozapine when the neutrophil count is greater the $1.0 \times 10^9/L$. Haematologist and CPMS consultation is required to approve adjustment of the neutrophil and WCC reference ranges.

5.2.1 Caffeine intake:

- Caffeine **increases** Clozapine levels which may result in toxicity. It is important to share this information with the consumer/relevant others and to document baseline smoking and caffeine intake habits which must be updated at each clinical review. Any change in smoking and caffeine consumption must be reported to the prescribing clinician.

5.2.2 Smoking, E cigarettes and Vaping

- Cessation of smoking **increases** Clozapine levels which may result in toxicity
- Cigarette smoke **decreases** Clozapine levels (poly-hydrocarbons in the tar affect Clozapine levels)
- Nicotine replacement therapy (NRT) does not affect Clozapine levels
- Vaping may affect Clozapine metabolism as some vape products containing combustible products can induce CYP450 enzymes.
- Switching from Cigarettes to vapes can affect clozapine metabolism
- Health professionals should monitor plasma clozapine levels in consumers who switch from smoking to vaping and adjust dosages as needed.

6. Side Effects:

The monitoring of side-effects occurs at each review of the consumer. In the community, the consumer is asked to complete the self-assessment form (PAKMR9.1.1) and this is reviewed by the Registered Nurse (RN) and treating doctor to seek greater clarity around the answers. If

side-effects are evident, the severity is to be assessed and the need to increase the frequency of future monitoring advised and agreed with the consumer.

During monitoring, attention should be paid to flu like symptoms and constipation. In the event that an unusual adverse effect is noted, this should be reported to the relevant Clozapine monitoring service.

Consumers will be encouraged to complete the verified Clozapine side-effect rating Glasgow Antipsychotic Side-effects Scale for Clozapine (GASS-C)

The most common side effects related to Clozapine are:

Common > 1%	Infrequent 0.1-1%	Rare < 0.1%
• Drowsiness • Hypersalivation • Constipation • Seizures • Headache • Tachycardia • Hyperpyrexia • Hepatitis • Neutropenia • Vomiting • Urinary Incontinence • Nocturnal Enuresis	• Myocarditis • Agranulocytosis • Eosinophilia • Priapism • EPSE	• Cardiomyopathy • Hypertension • Myoclonic Jerks • Interstitial nephritis • Respiratory arrest • Fulminant hepatic Necrosis • Polyserositis

7. Blood testing

A full blood picture (FBP) including White Cell Count (WCC) and Absolute Neutrophil Count (ANC) is required for all consumers prescribed Clozapine. The timing of these tests depends on the phase of treatment.

Weekly testing is required during Clozapine initiation and throughout the first 18 weeks of treatment or following any treatment interruption lasting > 4 weeks.

Monthly testing applies once the consumer has completed the initial 18 weeks and is considered on maintenance phase.

Bi-Weekly testing occurs whenever there is an amber result and continues until a green result is achieved.

NB: Blood results older than 48 hours from the appointment date are not valid for Clozapine prescription purposes and cannot be used to authorise dispensing.

Clozapine blood results monitoring system is divided into 3 defined ranges for WCC and ANC as in the table below:

	x10 ⁹ /L		
<u>State</u>	<u>WBC</u>	<u>Neutrophil</u>	<u>Outcome</u>
<u>Green</u>	≥ 3.5	≥ 2	<u>Continue with regular blood tests</u>
<u>Amber</u>	≥3.0 and <3.5	≥ 1.5 and <2.0	<u>Twice weekly blood tests required</u>
<u>Red</u>	<3	< 1.5	<u>Cease therapy immediately</u>

If the consumer's blood results return in the green range, they can be prescribed the full amount of Clozapine **until their next blood test**.

If the consumer's blood results return in the amber range, Clozapine therapy can continue but blood testing and clinical review must be increased to **TWICE a week** until their blood results return to the green range. They must be prescribed a reduced quantity of medication during this amber period until their next blood test (i.e. 2 – 3 days).

If the consumer's blood results return in the red range, Clozapine therapy must be discontinued immediately and reported to the relevant patient monitoring system; Clopine Central™ or eCPMS™.

Clopine Central™ offers haematology advice for any consumer on Clozapine therapy. The Clopine® haematologist can assist with any haematology queries as well as altering WCC and ANC thresholds for specific consumers (e.g., consumers on active chemotherapy or benign ethnic neutropenia where thresholds may need to be lowered).

An on-call haematologist at Clopine Central™ can be contacted on 1800 656 403 for advice.

eCPMS™ offers information via a cardiovascular referral for a patient on Clozaril who needs cardiology input. Medical information can be contacted on 1800 931 383.

Additional blood tests including Clozapine levels may be requested based on additional monitoring requirements or at the discretion of the treating doctor.

8. Physical health screening

Overall monitoring, review and referral to appropriate services for physical health is the responsibility of the consumer's lead professional (Responsible Medical Officer or Care Coordinator in Community and the allocated treating teams RMO in Inpatients). The RN undertaking the Clozapine nurse role and facilitating the clinic will assist in the screening of weight, girth, calculation of body mass index (BMI), BP, and TPR.

Metabolic screening should be undertaken in accordance with the AKG [Metabolic Screening Policy](#).

Physical Health medications should be reviewed by GP and dispensed by the nominated Community Pharmacy. However, if a significant delay in accessing GP care is expected and a need for treatment is identified during screening, the treating doctor may initiate medication. Ongoing follow up will remain the responsibility of the GP.

Correspondence with the identified GP should occur at least every three months. Completion of a GP letter must be recorded using the Clozapine Monitoring Form Part B. This letter should include any specific request or instructions regarding ongoing physical health monitoring.

Additional blood tests and physical observations required as part of metabolic screening

- **Vital Signs:** Blood pressure (Lying or sitting and standing), Pulse, Temperature, Respiratory Rate should be assessed at every review.
- **Urea and Electrolytes:** as clinically indicated, including baseline, 3 months, 6 months and then 6 monthly.
- **Weight, BMI and abdominal circumference:** Assess at every review
- **Fasting serum lipids (including total cholesterol, LDL, HDL, and Triglycerides):** as clinically indicated, including baseline, 3 months, 6 months and then 6 monthly.
- **HbA1c:** baseline, then as clinically indicated
- **Liver Function test:** as clinically indicated, including baseline, 3 months, 6 months and then 6 monthly.
- **Clozapine/Norclozapine Levels** – 5 weeks, 12 weeks and then 3 monthly. Serum Clozapine levels should be obtained 12 hours after the last dose. Monitoring is encouraged to assess non-compliance, poor response to adequate doses, effect of dose increases, adverse side effects, signs of toxicity, consideration of augmentation to Clozapine or other changes to medication, significant change to smoking or caffeine intake, deterioration of mental state, presence of liver disease.

Consumers whose Clozapine level provides adequate therapeutic effect below 350ug/L (literature evidence of best efficacy), should have this recorded on their management plan/in health care record.

- **Beta HCG** – pregnancy status and breastfeeding status (females).
- **Cardiac Monitoring**
- **Troponin** – baseline, weekly for first 6 weeks of initiation (weeks 1-6), 12 weeks then 6 monthly.
- **C reactive Protein (CRP)** – baseline, weekly for first 6 weeks of initiation (weeks 1-6), 12 weeks then 6 monthly.
- **Echocardiogram** – baseline, 3 months, years 1, 2, 5 and then 5 yearly intervals. Repeat echo as clinically indicated i.e., resting tachycardia, tachypnoea, shortness of breath or hypotension.
- **ECG** – as clinically indicated, including baseline until first 6 weeks, 12 weeks, 6 months, 12 months and then annually.

NB Clinical judgement and or emerging signs and symptoms will determine appropriate intervals of monitoring outside the recommended parameters. Consumers on high doses of Clozapine may require more frequent monitoring

9. Polypharmacy

Additionally, consumers may be taking other medications which require monitoring including:

- Amisulpride, Risperidone, Olanzapine > 20mg, Paliperidone – prolactin
- Quetiapine –TSH
- Carbamazepine – TSH, serum Carbamazepine
- Lithium carbonate – TSH, serum lithium
- Sodium Valproate – Prothrombin Time, Valproic acid level

The above list is not exhaustive and all medication should be reviewed appropriately.

Concomitant use of depot antipsychotics is discouraged but may be considered on an individual basis to achieve optimal treatment.

10. Overdue blood test results

The relevant monitoring service automatically alerts the service, registered doctors and nominated pharmacies to any overdue blood tests. The RN will alert the treating doctor (IPU)/ Care coordinator (Community) so follow up can be arranged. The relevant monitoring services should be notified if a consumer refuses to allow any further blood testing to be performed.

11. Therapy Interruption

The Clozapine monitoring protocol must be followed if a patient's blood test is missed or therapy is interrupted.

Time since last Clozapine dose	Clozapine dose	Blood test monitoring
<48 hours	Restart at normal dose of Clozapine	No changes to monitoring
≥ 48 hours - ≤72 hours	Restart Clozapine at 12.5mg. Consideration for inpatient admission.	No changes to monitoring
≥72 hours- <4 weeks	Restart Clozapine at 12.5mg and retitrate in IPU	Patient on weekly monitoring: Continue weekly blood tests for at least 6 weeks or for as long as a total of 18 weeks of monitoring has been reached, whichever is longer. Patient on monthly monitoring: Weekly blood tests for 6 weeks then monthly

		thereafter if no problems detected,
≥4 weeks	Restart Clozapine at 12.5mg and retitrate in IPU	Recommence as new patient, weekly monitoring for 18 weeks

12. Clozapine Discontinuation Considerations

Abrupt cessation of clozapine carries the risk of cholinergic rebound which may present as profuse sweating, nausea, vomiting, diarrhoea and headache. Additionally, there is an increased risk of relapse or rebound of psychosis therefore, close monitoring of the consumers mental state is essential.

To minimise these risks, a gradual tapering of Clozapine is recommended under the advise and direction of the treating doctor.

Once Clozapine has discontinued, the relevant Clozapine monitoring service must be notified to ensure records are updated.

13. Post monitoring blood tests Post Discontinuation

13.1 Weekly Monitoring:

Testing must occur at time of discontinuation and continue weekly for at least 4 weeks

13.2 Monthly Monitoring:

Testing must occur close to time of discontinuation and then four weeks later.

13.3 Twice-weekly monitoring due to amber result:

Patients who are being monitored twice weekly due to blood results in 'amber' range and who have discontinued clozapine should continue twice weekly blood tests until blood results return to 'green' range. Once in 'green' range, continue post monitoring blood tests to reflect prior monitoring schedule i.e., weekly monitoring or monthly monitoring. See above.

14. Pharmacy Services and Dispensing Protocol

The Clozapine Clinic will work closely with AHS pharmacy colleagues and community pharmacies. NB Clozaril® is only currently supplied by AHS pharmacy

The pharmacy role/responsibilities include:

Pharmacy will only dispense on receipt of a green or amber result:

- Review prescriptions and ensure they meet all PBS, state and Clozapine monitoring requirements before dispensing.
- Ensure each prescription is accompanied by FBP and entered into the appropriate Clozapine patient monitoring system (Clopine Central™ or eCPMS™) prior to dispensing.
- Dispense and supply clozapine as per patient monitoring status and frequency as per Clozapine monitoring system requirements.

- Monitor patient total drug therapy and identify potential drug interactions, dosage changes, if additional prescriptions for other medications are required and suggest clinical recommendations when necessary.
- Liaise with clinical staff when appropriate e.g. issues in respect of dispensing of Clozapine. Applying for extra dispensation and contacting Clopine about therapy events is the responsibility of the prescriber and not pharmacy.

15. Supplying of additional Clozapine to consumers

Consumers on Clozapine may only be prescribed enough Clozapine supply to last them until their next blood test.

- 7 days' supply for patients on weekly blood monitoring.
- 28 days' supply for patient on monthly blood monitoring.
- 2-3 days' supply if a patient's WCC or ANC result has returned in the amber range.

Clopine Central™ may authorise Centres to dispense one-off additional medications to consumers in cases of public holidays, travel, etc. Medical staff can complete the dispensation request online at www.clopine.com.au or by faxing the Additional Clopine® Treatment Dispensation form to Clopine Central on 1800 657 454.

The Clozaril Patient Monitoring System™ (eCPMS™) provides the same service for consumers prescribed Clozaril® www.ecpms.com.au Fax 1800 501 768

It is at the decision of the treating medical practitioner to decide if additional supply is appropriate for the patient. This should also include consideration if the patient will be compliant with their mandatory blood tests.

16. Commencing Clozapine (or recommended after interruption of more than 28 days)

Clozapine must be commenced in an inpatient facility due to the requirement for close physical monitoring and the dose titration that is required during the first two weeks of treatment.

There is an expectation that all consumers will have the following baseline investigations done prior to admission for Clozapine commencement/re-commencement:

- Full Blood Count (FBC) (within 10 days prior to commencement) including White Blood Cell (WBC) count and Neutrophil Cell Count (NCC)
- Urea and Electrolytes
- Blood group (BG)
- Fasting glucose (BGL)
- Fasting lipids
- Liver Function Tests (LFT)
- C-reactive protein (CRP)
- Troponin (TROP)
- Echocardiogram (Echo)
- 12 lead Electrocardiogram (ECG)
- Physical examination:

- Weight + Height (calculate BMI)
- Waist circumference
- Heart rate (HR)
- Temperature
- Blood Pressure (BP) (lying / sitting and standing)
- Respiratory rate
- Record current smoking status, caffeine intake, lifestyle and diet
- Investigation potential problems with prescribed, purchased, or herbal medication, illicit drugs and alcohol

Mental health clinicians must practice within the Mental Health Act (MHA) 2014 and the Charter of Mental Health Care Principles (MHA 2014)

17. Inpatient commencement monitoring

17.1 Medical Officer (MO) to:

- Prescribe Initiation of Clozapine on the WA Adult Clozapine Initiation and Titration form PAKMR170.4
 - Refer to the chart for all dosing and monitoring guidelines
- Ensure that all consumers complete the EMHS Consent to Clozapine Treatment Form EMR1270 prior to commencement
- Commence a Clozapine Monitoring Form Part A and B booklet on initiation to ensure records of monitoring and side effect assessments are completed. Chart to be completed once per week whilst consumer is on weekly bloods - usually on day of blood test. Refer to chart for side effect assessment guides and all other monitoring requirements.

17.2 Nursing staff to:

- Record initial consumer vital signs as per the WA Adult Clozapine Initiation and Titration Chart (PAKMR170.4) and also concurrently on the Adult Observation and Record Chart (AORC)

18. Inpatient admission of community patient on regular Clozapine

To ensure the continuation of required monitoring during an inpatient admission for a patient who has already be titrated on Clozapine the Care Coordinator is to confirm the below information is present in the patient medical record when such a patient is admitted:

- Medication list, ensuring the Clozapine prescription is current
- Copy of the Consent to Clozapine Treatment Form EMR0270
- Copy of inpatient Clozapine Monitoring Form Part A and B booklet

19. Accessing Clozapine afterhours

Clozapine can be accessed from AHS pharmacy for patients who are admitted at AHS inpatient wards or in ED after hours through the after hours' nurse manager (AHNM), after hours' clinical nurse specialist (AHCNS) or after hours' mental health clinical nurse specialist (AHMHCNS). Please see [After Hours Clozapine Acquisition Procedure](#) for more information.

20. Discharge from inpatient admissions to community mental health team

The Allocated Clinician is to:

- Ensure the ward specific discharge process is followed and complete all documentation in accordance with existing local policies
- Contact CMHS taking over care of consumer and confirm Care Coordinator. Document outcome in Care Transfer Summary.
- Ensure the following Clozapine specific information is available in the Digital Medical Record (DMR):
 - Copy of WA Adult Clozapine Initiation and Titration Chart (if consumer's Clozapine initiated during admission)
 - Copy of inpatient Clozapine Monitoring Form Part A and B booklet
 - Either a copy of or date of latest ECHO, ECG and most recent blood results
 - Copy of the Consent to Clozapine Treatment Form (for consumers who've commenced Clozapine if completed/available)
- Confirm when next blood test is due (date)
- Communicate with CMHS to ensure appointment time is appropriately booked and ensure sufficient clozapine doses are supplied to the patient until appointment date.
- Provide consumer with blood test request form.

21. Community Clozapine review

The allocated Clozapine nurse will liaise with administration staff in the community to enter a recurring appointment with the identified doctor for review at the Clozapine clinic and ensure that the consumer has a pathology request form in order to get bloods taken.

- The Consumer will complete the GASS-C side effect assessment at each visit.
- The consumer will be reviewed by the treating doctor or Clozapine coordinator for identified consumers in the community to review GASS-C information and assess for sign and symptoms of infection, Clozapine induced side-effects, mental wellness and risk
 - Weekly for the first 18 weeks
 - 28 days
 - On re-titration or following therapy interruption for 6 weeks
- Record keeping is completed on PAKMR 6.9.1 (side effect assessment), form part of the medical record, Progress note DMR attendance is entered into PSOLIS.

21.1 The RN will:

- Assist in obtaining blood results from the nominated pathology centre
- Undertake physical observations for all consumers including BP, TPR, weight, waist circumference and unless consent is withdrawn by the consumer.
- Assisted in completing the self-assessment form (PAKMR 6.9.1)
- Monitor side effects and inform relevant medical staff
- Coordinate all Clozapine related issues for consumers and carers

- Monitor Clopine Central™ and eCPMS™ alerts and coordinate with medical team and care coordinators.
- Monitor therapy interruptions at additional clinic appointments by performing physical observations.

21.2 Medical staff will:

- Be registered with the identified monitoring services
- Undertake MSE and Risk Assessment at medical review
- Review all current medications and compliance (including those prescribed by a GP or other specialist)
- Identify if consumer has any excess Clozapine or other medications available to them prior to issuing a new prescription (supply of Clozapine should not exceed 7- or 28-days dependent upon monitoring)
- Review Gass-C
- Review and acknowledgment of consumers last blood test noting that it is < 48 hours old and falls within a Green or Amber range
- Issue pathology request for a minimum of full blood count (FBC) and any additional tests which are required. NB there is a delay for Clozapine serum levels due to batch testing and these may not be available for the next appointment).
- Write a valid PBS prescription for the appropriate quantity of Clozapine to be supplied
- Provide admin with the prescription, Clozapine blood count form and original blood test result to be faxed/emailed to the nominated pharmacy.
- Inform pharmacy/GP of any dose changes via a Pharmacy Notification form confirming receipt of same.
- Monitor interruptions or cessations to treatment and ensuring increased pathology requests and reduced prescribing are adhered to when indicated.
- Communicate with consumers GP at maximum of 3 monthly intervals highlighting any concerns with physical health and current medications
- Notify Clopine Central™ or eCPMS™ immediately if Clozapine therapy is discontinued or therapy event occur
- Reporting all adverse events to Clopine Central™ or eCPMS™

22. Outpatient DNA

In the event that a consumer fails to attend their appointment, the medical team will initiate the Community DNA procedure [Scheduling, Rescheduling and Non-Attendance \(DNA\) of Appointments in Community Mental Health Clinics](#)

Additionally, the nurse facilitating the Clozapine clinic will assist in:

- Making an alternative appointment within the Clozapine protocol time frame
- If the consumer is unable to attend their appointment the Clozapine nurse or care coordinator will facilitate a telehealth appointment via health direct to support and assess the consumer and to organise continuation of medication supply (if appropriate).

- In the event of repeated non-attendance or difficulty in contacting the consumer escalation to the Consultant Psychiatrist for treatment plan needs to occur. The supplying pharmacy should also be informed

23. Compliance monitoring

Compliance with the requirements of this procedure will be monitored via the Mental Health Safety and Quality Committee.

Compliance will be assessed via the following mechanism.

- Annual Clozapine audit of health care record including consent to treatment, commencement, monitoring, prescribing and management of Clozapine.
- Audit to be performed through Mental Health Documentation Audit Survey Tool
- Review of Datix CIMS and complaints relating to Clozapine

24. Policy context

The following documents are required to give effect to this procedure:

- [WA Mental Health Act 2014](#)
- [WA Charter of Mental Health Care Principles.](#)
- [EMHS Antipsychotic Polypharmacy and High Dose Antipsychotic Prescribing Procedure](#)
- [AKG Medication Management](#)
- [Adverse Drug Reaction and Clinical Alert Procedure](#)
- [PBS Section 100 - Highly Specialised Drugs Program](#)
- [AKG Medication Policy](#)
- [AKG After Hours Clozapine Acquisition Procedure](#)
- [Metabolic Screening Procedure](#)
- EMR1099 Clozapine Monitoring Form - Part A
- EMR101 Clozapine Monitoring Form- Part B

25. Relevant standards

[Chief Psychiatrist's Standards for Clinical Care](#)

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.04, 2.06, 2.07, 2.10
- Standard 4 – Action 4.03, 4.10, 4.15
- Standard 5 – Action 5.07, 5.10, 5.11
- Standard 6 – Action 6.03, 6.07, 6.08, 6.09, 6.10
- Standard 8 – Action 8.04, 8.05, 8.06, 8.08, 8.09, 8.12, 8.13

26. Glossary

The following definitions are relevant to this procedure:

Term	Definition
ANC	Absolute Neutrophil Count

Term	Definition
BP	Blood Pressure
CC	Care Coordinator
Clopine Central™	Clopine® Patient monitoring service
eCPMS™	Clozaril® Patient Monitoring Service
ECG	Electrocardiogram
FBP	Full Blood Picture
IPU	In Patient Unit
NRT	Nicotine Replacement Therapy
TGA	Therapeutic Goods Administration
TPR	Temperature, Pulse, respiration
TSH	Thyroid Stimulating Hormone

27. Document contact

Enquiries relating to this procedure may be directed to:

Title: Team Leader
 Department: Clinical Treatment Team
 Email: alison.strickland@health.wa.gov.au

28. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2.0	01/10/2025	01/10/2028	Significant changes throughout documents.

29. Authorisation

This procedure has been authorised and issued by:

Authorised by	Shannon Stuart, Service Director, Mental Health
Authorisation date	22 September 2025
Published date	01 October 2025